

• Patient name: **Laura Senne Moretti**

• Gender: **Female**

• Date of birth: **04-08-1994**

• Sample code: **BOX03464AANUT**

• Sample date: **09-06-2026**

• Date of the results: **15-06-2026**



Fagron NutriGen

Patient report



Fagron NutriGen

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Patient identification data

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Patient's name and last name —●— **Laura Senne Moretti**
Date of Birth —●— **04-08-1994**
Gender —●— **Female**
Sample type —●— **Buccal mucosa**
Sample code —●— **BOX03464AANUT**
Sample date —●— **09-06-2026**
Report date —●— **16-06-2026**

Doctor's name and last name —●— **Ketone Clinical Laboratory**
Email address —●— **info@ketone.ae**
Telephone —●— **+971 4 892 2499**

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08226 Terrassa, Barcelona (Spain)



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FGMS-Nutri



In Vitro Diagnostic MedicalDevice



8437024682FGMS-NutriVC

Genetic results overview

2

2.1. Morphological genetics for weight control

38.6% Medium-high genetic predisposition to being overweight. In case of overweight or obesity, it is caused mainly by inherited genetics. Following the recommendations of this DNA analysis will improve outcomes.



Genetic risk of overweight/obesity

Risk of rebound weight gain

Risk of increased BMI

Basal metabolic rate (burn calories at rest)

Weight loss capability during diet interventions

2.2. Behavioural genetics in food intake

46.58% Medium-high dysregulation of food intake behaviour. High predisposition to being overweight. Strategies to improve satiety should be considered.



Appetite and anxiety risk

Satiety: Feeling Full

2.3. Flavour sensitivities

51% Slightly increased flavour sensitivity. No important impact on the diet is predicted.



Bitter taste sensitivity

Salt sensitivity

Sweet flavour preference

2.4. Fat metabolism

16.61% Highly negative fat burning capacity. It would be recommended to greatly decrease the fat intake.



Response to monounsaturated fats (MUFAs)

Response to polyunsaturated fats (PUFAs)

Response to fat intake to improve the HDL levels

2.5. Lipid metabolism

48.92% Affected lipid metabolism. Cholesterol and triglyceride levels may show irregular results in blood analyses. Specific LDL or HDL treatments would be recommended. Increased cardiovascular risk.



Predisposition to reduced HDL levels

Predisposition to increased levels of triglycerides

Predisposition to increased oxidation of LDL

Risk of increased cholesterol LDL levels

Risk of unbalanced Triglycerides/HDL ratio

2.6. Carbohydrate metabolism

19.13% Highly negative carbohydrate metabolism: Carbohydrate intake will lead to dysregulation in cholesterol levels and also to increased calorie and fat intake. Eliminating refined carbohydrates is urgent; move to wholegrain carbohydrates and reduce the quantity.



Capability to digest starchy food

Refined carbohydrate sensitivity

Carbohydrates and HDL levels predisposition

Carbohydrates and LDL levels

INDICATIONS

■ Positive effect

■ Medium-positive effect

■ Medium-negative effect

■ Negative effect

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Genetic results overview

2

2.7. Glucose metabolism

30.36% Medium-high dysregulation of glucose metabolism. Intake of refined sugar and carbohydrates will be dangerous. High risk of developing Type-II diabetes.



Risk of increased glucose levels in plasma after fasting
Risk of insulin resistance
Risk of Type-II diabetes

2.8. Efficacy of exercise

25.56% Low-medium efficacy of exercise to reduce body fat and regulate cholesterol levels. Intensive dietary interventions may be the best option.



Benefits from endurance exercise for improving HDL levels
Exercise to reduce body fat

2.9. Detoxification imbalances

72.19% Slightly reduced detoxification capacities. Try to decrease toxin exposure and intake.



Antioxidant capability

2.10. Intolerance

LACTOSE



Lower risk of lactose intolerance.

GLUTEN



Carrier of celiac disease risk variants.
Try to reduce or avoid gluten-containing food (consult your specialist before making any dietary changes).

FRUCTOSE



Lower risk of fructose intolerance.

CAFFEINE



Very slow caffeine metabolism speed: caffeine will last longer in the body. Be careful with excess caffeine.

ALCOHOL



Normal risk of alcohol toxicity due to a normal metabolism.

INDICATIONS

Positive effect **Medium-positive effect** **Medium-negative effect** **Negative effect**

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Genetic results overview

2

2.11. Hormones

LEP

Predisposition to increased levels of leptin leading to leptin receptor saturation that may lead to increased risk of overeating and lower fat burning.

NAMPT

High predisposition to increased levels of circulating visfatin augmenting considerably the risk of an altered inflammatory response.

GHSR

Predisposition to normal ghrelin receptor (GHSR) expression. Circulating levels of this hormone and the sensation of hunger are increased.

ADIPOQ

High predisposition to lower adiponectin plasma levels that leads to an increased inflammation process, lipid abnormalities and insulin resistance.

ADIPOQ

High predisposition to lower adiponectin plasma levels that leads to an increased inflammation process, lipid abnormalities and insulin resistance.



2.12. Inflammation

TNF-α

Predisposition to average levels of TNF-alpha.

IL-6

Predisposition to highly increased levels of IL-6. Pro-inflammation.

IL-10

Predisposition to higher levels of the anti-inflammatory cytokine IL-10.



INDICATIONS

Positive effect

Medium-positive effect

Medium-negative effect

Negative effect

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Genetic results overview

2

2.13. Vitamins

Vitamin A

A

■ **Normal vitamin A metabolism.**
Ensure daily recommended intake.

Vitamin B6

B⁶

■ **High risk of vitamin B6 deficiency.**
Increase daily vitamin B6 intake.
Supplementation should be evaluated.

Vitamin B9
(folate)

B⁹

■ **Low risk of folate deficiency.**
Ensure daily recommended intake.

Vitamin B12

B¹²

■ **Low risk of vitamin B12 deficiency.**
Ensure daily recommended intake.

Vitamin C

C

■ **Normal vitamin C metabolism and levels.** Ensure daily recommended intake.

Vitamin D

D

■ **Low risk of Vitamin D deficiency.**
Ensure daily recommended intake.

Vitamin E

E

■ **High risk of Vitamin E deficiency.**
Ensure daily recommended intake. Supplementation strategies would be recommended.

2.14. Minerals

CALCIUM



Calcium malabsorption risk

■ **Low inherited risk of calcium malabsorption.**

Predisposition to dysregulated calcium levels

■ **Slightly increased risk of dysregulated plasma calcium levels.**

IRON



Risk of iron overload

■ **No additional risk of iron overload.**

Risk of low iron plasma levels

■ **No additional inherited risk of low iron levels.**

MAGNESIUM



Predisposition to dysregulated magnesium levels

■ **Medium risk of dysregulated plasma magnesium levels.**

SELENIUM



Predisposition to dysregulated selenium levels

■ **No additional risk of dysregulated plasma selenium levels.**

SALT



Sodium sensitivity

■ **Slightly increased sodium sensitivity: moderately increased blood pressure risk due to salt consumption.**

INDICATIONS

■ **Positive effect**

■ **Medium-positive effect**

■ **Medium-negative effect**

■ **Negative effect**

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Genetic results overview

2

2.15. Effectiveness of diets

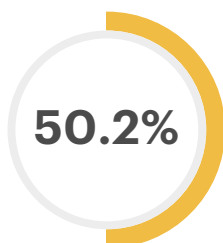


■ 75% - 100%
High efficacy

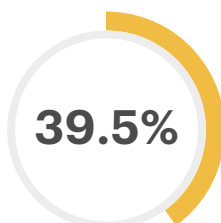
■ 50% - 75%
Medium-high efficacy

■ 25% - 50%
Medium efficacy

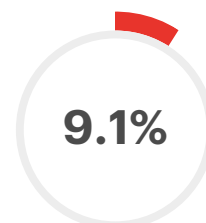
■ 0% - 25%
Low efficacy



Efficacy of low carbohydrate diets



Efficacy of low fat diets



Efficacy of low calorie diets

Recommended nutritional plan

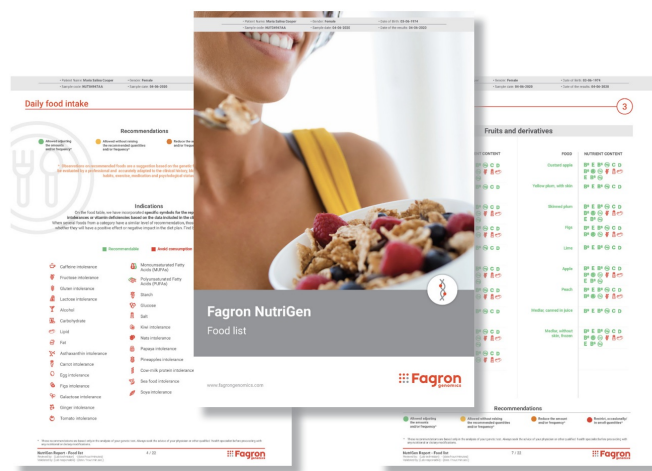
3



The most effective diet for your patient, after the genetic analysis, would be

LOW IN CARBOHYDRATES
INTEGRATED NUTRITIONAL PLAN

Check the **FOOD LIST**
recommended for you



Recommended supplements

4

The supplements recommended to combat overweight and ageing are divided into 3 phases

1 DETOX		2 INTESTINAL	3 SUPPLEMENTATION
Detoxification (oxidation) 30 days	Detoxification (conjugation) 30 days	Transportation Excretion 30 days	Prevention, maintaining optimal nutrition 3-6 months
• Glutathione (Reduced glutathione) • Silymarin • Quercetin • Selenium (Selenium yeast) • Resveratrol • Nicotinamide (Niacinamida) • Astaxanthin • Vitamin B2 (Riboflavine) • GreenSelectTM (Green tea extract)	• Magnesium • Vitamin B12 (Cyanocobalamin) • Glutathione (Reduced glutathione) • Silymarin • Quercetin • Gotu kola dry extract (Centella asiatica) • Resveratrol • Taurine • N-Acetylcysteine	You don't require this phase in your treatment	• Magnesium • Vitamin B12 (Cyanocobalamin) • Vitamin C (Ascorbic Acid) • Melatonin • Horsetail dry extract (Equisetum arvense) • Quercetin • Gotu kola dry extract (Centella asiatica) • Ginseng dry extract (Panax ginseng) • Selenium (Selenium yeast) • Resveratrol • Nicotinamide (Niacinamida) • Niacin (nicotinic acid)

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Complete genetic results

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2.1. Morphological genetics for weight control

GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Genetic risk of overweight/obesity MEDIUM-HIGH RISK OF OVERWEIGHT/OBESITY	MC4R	rs2229616	CC	
	SH2B1	rs7498665	AG	
	FTO	rs9939609	TA	
	FTO	rs1121980	GA	
	MC4R	rs17700633	GA	
Risk of rebound weight gain HIGH REBOUND EFFECT	ADIPOQ	rs17300539	GG	
Risk of increased BMI MEDIUM-LOW RISK OF INCREASED BMI	MC4R	rs12970134	GA	
	MC4R	rs17782313	TC	
	SH2B1	rs4788102	GA	
Basal metabolic rate (burn calories at rest) HIGH BURNER AT REST	FABP2	rs1799883	CC	
	LEPR	rs2025804	AA	
Weight loss capability during diet interventions SLOW WEIGHT LOSS	ACSL5	rs2419621	CC	

Complete genetic results

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2.2. Behavioural genetics in food intake

GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Appetite and anxiety risk INCREASED APPETITE AND ANXIETY RISK	COMT	rs4680	GG	■
	NMB	rs1051168	GG	■
	DRD2	rs1800497	GG	■
	MC4R	rs2229616	CC	■
	DRD2	rs6277	AA	■
Satiety: Feeling Full SLIGHTLY LOWER SATIETY	FTO	rs9939609	TA	■

2.3. Flavour sensitivities

GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Bitter taste sensitivity NORMAL BITTER TASTE SENSITIVITY	TAS2R38	rs1726866	GA	■
	TAS2R38	rs713598	CG	■
Salt sensitivity MEDIUM-LOW SALT SENSITIVITY	ACE	rs4343	GA	■
Sweet flavour preference HIGHLY INCREASED SWEET PREFERENCE	SLC2A2	rs5400	GA	■

INDICATIONS

■ Negative effect

■ Medium-positive effect

■ Positive effect

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Complete genetic results

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2.4. Fat metabolism				
GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Response to monounsaturated fats (MUFAs) VERY LOW MUFA METABOLISM	ADIPOQ	rs17300539	GG	
Response to polyunsaturated fats (PUFAs) LOW PUFA METABOLISM	PPARG	rs1801282	CC	
	FADS1	rs174547	CC	
Response to fat intake to improve the HDL levels VERY LOW BENEFITS TO IMPROVE HDL	LIPC	rs1800588	CC	

INDICATIONS

Negative effect

Medium-positive effect

Positive effect

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Complete genetic results

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2.5. Lipid metabolism				
GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Predisposition to reduced HDL levels NORMAL HDL LEVELS	APOA5	rs662799	AA	
	CETP	rs5883	CT	
Predisposition to increased levels of triglycerides HIGHLY INCREASED TRIGLYCERIDES	PPARG	rs1801282	CC	
Predisposition to increased oxidation of LDL SLIGHTLY INCREASED LDL OXIDATION	APOB	rs676210	GA	
Risk of increased cholesterol LDL levels INCREASED LDL LEVELS	CELSR2	rs12740374	GG	
	HNF1A	rs2650000	CC	
	LDLR	rs6511720	GG	
	ABCG8	rs6544713	TC	
Risk of unbalanced Triglycerides/HDL ratio SLIGHTLY INCREASED TG/HDL RATIO	HMGCR	rs3846663	CT	

INDICATIONS

 Negative effect

 Medium-positive effect

 Positive effect

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Complete genetic results

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2.6. Carbohydrate metabolism				
GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Capability to digest starchy food	AMY1-AMY2	rs11577390	CT	
INCREASED STARCH DIGESTION	AMY1	rs4244372	TT	
Refined carbohydrate sensitivity				
VERY HIGH CARBOHYDRATE SENSITIVITY	FABP2	rs1799883	CC	
Carbohydrates and HDL levels predisposition				
HIGH RISK OF HDL DYSREGULATION	KCTD10	rs10850219	GG	
Carbohydrates and LDL levels				
HIGH RISK OF LDL DYSREGULATION	MMAB	rs2241201	CG	

INDICATIONS

 Negative effect

 Medium-positive effect

 Positive effect

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Complete genetic results

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2.7. Glucose metabolism				
GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Risk of increased glucose levels in plasma after fasting	PLIN1	rs2289487	CT	
	GHSR	rs490683	GG	
MEDIUM-HIGH RISK OF HIGH GLUCOSE LEVELS				
Risk of insulin resistance	PPARG	rs1801282	CC	
	ADIPOQ	rs17300539	GG	
	TCF7L2	rs7903146	CT	
	FTO	rs9939609	TA	
	FTO	rs1121980	GA	
MEDIUM-HIGH INSULIN RESISTANCE				
Risk of Type-II diabetes	PPARG	rs1801282	CC	
	PLIN1	rs2289487	CT	
	TCF7L2	rs7903146	CT	
	FTO	rs9939609	TA	
	MC4R	rs17700633	GA	
	CDKN2A/B	rs10811661	TT	
	KCNQ1	rs2237892	CC	
	CDKN2A, CDKN2B	rs2383208	AA	
	CDKAL1	rs7756992	AA	
	TCF7L2	rs7901695	TC	
MEDIUM-HIGH DIABETES TYPE-II RISK				

INDICATIONS

Negative effect

Medium-positive effect

Positive effect

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Complete genetic results

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2.8. Efficacy of exercise



GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Benefits from endurance exercise for improving HDL levels VERY LOW BENEFITS FROM EXERCISE FOR IMPROVING HDL	PPARD	rs2016520	TT	
Exercise to reduce body fat MEDIUM-HIGH BENEFIT FROM EXERCISE TO REDUCE FAT	FTO	rs9939609	TA	
	FTO	rs1121980	GA	
	LIPC	rs1800588	CC	
	LEP	rs7799039	GA	

2.9. Detoxification imbalances



GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Antioxidant capability SLIGHTLY REDUCED ANTIOXIDANT CAPABILITY	GPX1	rs1050450	GA	
	NQO1	rs1800566	GG	
	COMT	rs4680	GG	
	SOD2	rs4880	GG	
	CYP1B1	rs1056836	GG	
	CYP1A1	rs1048943	TT	
	GSTP1	rs1695	AG	

INDICATIONS

Negative effect **Medium-positive effect** **Positive effect**

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Complete genetic results

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2.10. Intolerance					
	GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
	Lactose intolerance risk	MCM6	rs182549	CT	
	LOWER RISK OF LACTOSE INTOLERANCE	MCM6	rs4988235	GA	
	Risk of celiac disease MEDIUM-HIGH RISK OF CELIAC DISEASE	HLA-DQ2.5	rs2187668	CC	
		HLA-DQ7.5	rs4639334	GG	
		HLA-DQ2.2	rs2395182	GT	
		HLA-DQ2.2	rs4713586	AG	
		HLA-DQ8	rs7454108	TT	
		HLA-DQ2.2	rs7775228	CC	
	Fructose intolerance risk	ALDOB	rs1800546	CC	
	LOWER RISK OF FRUCTOSE INTOLERANCE	ALDOB	rs76917243	GG	
	Caffeine metabolism	CYP1A1	rs2470893	CC	
	SLOW CAFFEINE METABOLIZER	CYP1A2	rs762551	CA	
	Alcohol metabolism	ALDH2	rs671	GG	
	NORMAL ALCOHOL METABOLISM				

INDICATIONS

 Negative effect

 Medium-positive effect

 Positive effect

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Complete genetic results

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2.11. Hormones					
	GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
	Leptin				
	HIGH RISK OF DECREASED LEPTIN LEVELS	LEP	rs7799039	GA	
	Visfatin				
	HIGH RISK OF INCREASED VISFATIN LEVELS	NAMPT	rs9770242	AA	
	Ghrelin				
	HIGH GHRELIN RECEPTOR (GHSR) EXPRESSION	GHSR	rs490683	GG	
	Adiponectin				
	HIGH RISK OF DECREASED ADIPONECTIN LEVELS	ADIPOQ	rs1501299	GG	
		ADIPOQ	rs2241766	TT	

2.12. Inflammation					
	GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
	TNF-α				
	LOW RISK OF DYSREGULATED TNF-α LEVELS.	TNF-α	rs1800629	GG	
	IL-6				
	HIGH RISK OF INCREASED IL-6 LEVELS	IL-6	rs1800795	GG	
	IL-10				
	LOW RISK OF DECREASED ANTIINFLAMMATORY CYTOKINE IL-10 LEVELS	IL-10	rs1800896	CC	

INDICATIONS

Negative effect

Medium-positive effect

Positive effect

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Complete genetic results

5

2.13. Vitamins					
	GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
A	Vitamin A LOW RISK OF VITAMIN A DEFICIENCY	BCMO1	rs12934922	AT	
		BCMO1	rs7501331	CC	
B6	Vitamin B6 HIGH RISK OF VITAMIN B6 DEFICIENCY	NBPF3	rs4654748	CC	
B9	Vitamin B9 (folate) MEDIUM-LOW RISK OF VITAMIN B9 (Folate) DEFICIENCY	MTHFR	rs1801133	GA	
B12	Vitamin B12 MEDIUM-LOW RISK OF VITAMIN B12 DEFICIENCY	FUT2	rs602662	GA	
C	Vitamin C LOW RISK OF VITAMIN C DEFICIENCY	SLC23A2	rs1279683	GA	
		SLC23A1	rs33972313	CC	
D	Vitamin D MEDIUM-LOW RISK OF VITAMIN D DEFICIENCY	GC	rs2282679	TT	
		CYP2R1	rs10741657	GG	
		NADSYN1	rs12785878	TT	
		CYP2R1	rs2060793	GG	
		NADSYN1	rs3829251	GG	
E	Vitamin E HIGH RISK OF VITAMIN E DEFICIENCY	INTERGENIC	rs12272004	CC	
		ZPR1	rs964184	CC	

INDICATIONS

Negative effect

Medium-positive effect

Positive effect

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Complete genetic results

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2.14. Minerals					
	GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Ca	Calcium malabsorption risk LOW RISK OF CALCIUM MALABSORPTION	CYP2R1	rs10766197	GA	
		GC	rs2282679	TT	
	Predisposition to dysregulated calcium levels SLIGHTLY INCREASED RISK OF DYSREGULATED PLASMA CALCIUM LEVELS	DGKD	rs1550532	GG	
		CYP24A1	rs1570669	AA	
		CASR	rs17251221	AA	
		CASR	rs1801725	GG	
		CARS	rs7481584	GA	
Fe	Risk of iron overload LOW RISK OF HEMOCHROMATOSIS	HFE	rs1800562	GG	
	Risk of low iron plasma levels LOW RISK OF DECREASED IRON LEVELS	TF	rs3811647	GG	
		TMPRSS6	rs4820268	GA	
Mg	Predisposition to dysregulated magnesium levels MEDIUM-HIGH RISK OF DYSREGULATED MAGNESIUM LEVELS	CASR	rs17251221	AA	
		TRPM6	rs11144134	TT	
		SHROOM3	rs13146355	GG	
		DCDC5	rs3925584	TT	
		MUC1	rs4072037	CT	
Se	Predisposition to dysregulated selenium levels NO ADDITIONAL RISK OF DYSREGULATED SELENIUM LEVELS	AGA	rs1395479	CC	
		SLC39A11	rs891684	GG	
	Sodium sensitivity MEDIUM-LOW SODIUM SENSITIVITY	ACE	rs4343	GA	

INDICATIONS

Negative effect

Medium-positive effect

Positive effect

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Complete genetic results

5

2.15. Effectiveness of diets



GENETIC RISK	MARKER	LOCUS	YOUR VARIANT	YOUR RESULT
Efficacy of low calorie diets VERY LOW EXPECTED BENEFIT FROM LOW-CALORIE DIET	PPARG	rs1801282	CC	
	ADIPOQ	rs17300539	GG	
	LEPR	rs1805134	TT	
	ACSL5	rs2419621	CC	
	ADRB2	rs1042714	GC	
Efficacy of low carbohydrate diets MEDIUM-HIGH EXPECTED BENEFIT FROM LOW-CARBOHYDRATE DIET	KCTD10	rs10850219	GG	
	MMAB	rs2241201	CG	
Efficacy of low fat diets MEDIUM-LOW EXPECTED BENEFIT FROM LOW-FAT DIET	PPARG	rs1801282	CC	
	GHSR	rs490683	GG	
	APOA2	rs5082	GG	
	SH2B1	rs7498665	AG	
	TCF7L2	rs7903146	CT	
	FTO	rs9939609	TA	

INDICATIONS

Negative effect

Medium-positive effect

Positive effect

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NutriGen Report - Patient

Reviewed by: (Lab technician) - (09-06-2026)

Validated by: (Lab responsible) - (15-06-2026)

Methodology

6

How were the genetic variants selected and evaluated?

This test was developed by a multidisciplinary team of medical doctors, geneticists, and programmers, following highest quality standards. In particular, an expert team specialized in the curation of genetic variants reviewed each variant to ensure that selection, interpretation and impact of variants in the algorithms are based on the highest scientific evidence.

The following selection criteria were applied for classifying genetic variants:

Level 1A: Annotation for a variant in medical society-endorsed or implemented in a major health system.

Level 1B: Annotation for a variant where the preponderance of evidence shows an association. The association must be replicated in more than one cohort with significant p-values, and preferably will have a strong effect size.

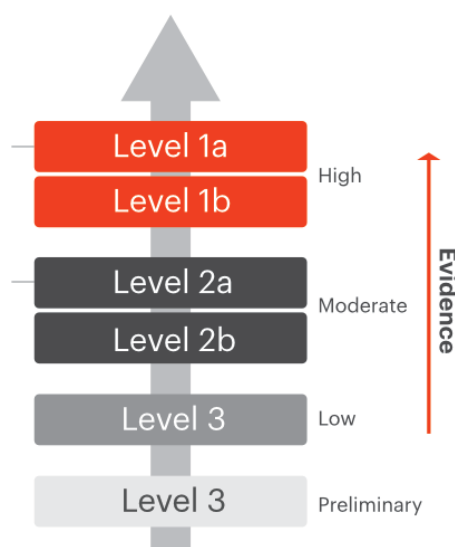
Level 2A: Annotation for a variant that qualifies for level 2B where the variant is within a Very Important known gene, so functional significance is more likely.

Level 2B: Annotation for a variant with moderate evidence of an association. The association must be replicated but there may be some studies that do not show statistical significance, and/or the effect size may be small.

Level 3: Annotation for a variant based on a single significant (not yet replicated) study or annotation for a variant evaluated in multiple studies but lacking clear evidence of an association.

Level 4: Annotation based on a case report, non-significant study or in vitro, molecular or functional assay evidence only.

Only genetic variants from level 1A to 2A were selected.



How was it analyzed?

The DNA was extracted from the buccal swab sample you provided and was analyzed by our clinical analysis laboratory. DNA was extracted using the KingFisher Flex® robotic extraction system (Thermo Fisher Scientific). The study of the genetic variants was performed by NGS (Next Generation Sequencing) using the Ion GeneStudio S5 system (Thermo Fisher Scientific).

References

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References

Scan the QR code to access our NutriGen report reference page.

* These recommendations are based only in the analysis of your genetic test. Always seek the advice of your physician or other qualified health specialist before proceeding with any nutritional or dietary modifications.

Together
we create the future
of personalizing medicine.

